MOOD-04 | Therapist only - all age groups

Thoughts and patterns that maintain withdrawal

1 | Purpose, fit and boundaries

Purpose of this unit

Explore one situation linking a personal interpretation, emotion and reduced action, and broaden possible understandings and responses. The aim is not to prove the thought wrong, but to explore what is known and what can be chosen.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Do not minimize harm, loss or real difficulties, or require positive thinking. Consider living conditions and resources alongside thoughts. This sheet is not for exploring a traumatic event or repeatedly checking for certainty.

Reusing existing material

Core-02 to Core-04 already cover links between situations, interpretations and responses. Here the focus is their effect on withdrawal during low mood and choosing a limited response; an existing record can be used.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | A conversation about one situation

Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

1 | Question for discussion

Which current situation suits a limited exploration?

2 | Question for discussion

What were the facts, and what was my interpretation?

3 | Question for discussion

How did interpretation and emotion affect withdrawal or avoidance?

4 | Question for discussion

What do I still need to know, and what other explanations are possible?

5 | Question for discussion

What chosen response might fit the actual conditions?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

Work with a small everyday event and one sentence the child chooses. Do not correct their words before understanding what they mean.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

Distinguish interpretation from actual social harm. Identifying details can be left off the sheet.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

Explore practical barriers too; an alternative interpretation does not resolve financial hardship, overload or a harmful relationship.

OA | Older adults

Respect life experience and losses. Do not replace the participant's interpretation with a companion's or assume that difficulty is due to age.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

Decide together whether practice between sessions is appropriate at all. Define one opportunity, boundaries, and desired support. Practice may take place only in the session. There is no required daily sequence or requirement to share the sheet with others.

At the next session

Check whether the conversation expanded choice and functioning or became an argument about thoughts. If exploration repeats without benefit, consider another tool.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

www.nice.org.uk/guidance/ng222/chapter/Recommendations
www.nice.org.uk/guidance/ng134/chapter/Recommendations

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